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Correct Answer

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Practice Exam - 1

Question 24 of 117



A 47-year-old man with history of HTN, prior tobacco use and non-small cell lung cancer presented to ER with 2-day history of worsening fatigue, malaise, dizziness, shortness of breath and palpitations. In the emergency room, the patient's vital signs are: heart rate of 120 beats per minute, blood pressure is 90/47 mmHg, and respiratory rate is 28/min. On examination he is ill appearing, crackles are noted to mid lung fields bilaterally. Cardiovascular exam reveals regular rhythm, 120 beats per minute, and an S3 gallop. Chest Xray is notable for vascular congestion changes. During the patient's examination salvos of non-sustained ventricular tachycardia are noted on telemetry.

Initiation of which medication can be associated with this patient's current presentation?

A. Carboplatin

 B. Pemetrexed

 C. Pembrolizumab

D. Gemcitabine

Commentary

References

Testing point: Recognize check-point inhibitor-associated myocarditis associated with check point inhibitors.

Answer: C. Pembrolizumab.

Pembrolizumab is a relatively new chemotherapeutic agent in the check-point inhibitor class. Cardiotoxicity is a very rare complication of immune checkpoint inhibitors, affecting between 1% and 10% of patients; the rate of cardiotoxicity is higher in patients taking a combination of ICIs. The precise mechanism of cardiotoxicity is unknown, but seems to involve CD4+ mediated T cell inflammation and immune infiltration of cardiac structures.

Answer A: Carboplatin is a chemotherapeutic agent used in many cancers, including non-small cell lung cancer. Carboplatin has no known cardiotoxicity.

Answer B: Pamextrexed is a chemotherapeutic agent used in non-small cell cancers. It is an antifolate anti-neoplastic agents and has no strong association with cardiotoxicity.

Answer D: Gemcitabine is a chemotherapeutic agent used in multiple cancer types. It is a nucleoside analog and not associated with cardiotoxic effects.

Myocarditis associated with check point inhibitors. Choices A, B, and D are not typically associated with myocarditis.

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